The CEO Business Challenge @ Cal Poly Pomona

Where Tomorrow's Leaders Tackle Today's Challenges

Inland Empire Health Plan (IEHP)

Analyzing Health-Care Service Models

Company Background

Founded in 1996, IEHP is a local initiative organized as a not-for-profit health plan serving residents of Riverside and San Bernardino counties. IEHP's mission and vision are focused on delivering optimal care and vibrant health for the community. With a membership of more than 1.4 million, IEHP is one of the top five largest Medicaid health plans and the largest not-for-profit Medicare-Medicaid public health plan in the country (Becker's Payer Issues, March 9, 2026). Inland Empire residents can join IEHP through Medi-Cal, Medicare Duals Special Needs Plan, and Covered CA. IEHP has held Health Plan Accreditation and Health Equity Accreditation from the National Committee for Quality Assurance (NCQA) for several years, with the most recent update in July 2026 (NCQA, July 15, 2026). Further, IEHP has been certified as a Great Place to Work (March 2026-March 2027) for a sixth year in a row.

More information is available at http://www.iehp.org

The Business Problem: Context for the Challenge

Medi-Cal serves more than 14 million Californians through two primary delivery models: **Managed Care** and **Fee-for-Service (FFS)**. It is the nation's largest Medicaid program covering nearly 3 in 8 California adults and half of the state's children, and IEHP is one of California's largest Managed Care Plans (MCP).

Effective January 1, 2027, the California Department of Health Care Services (DHCS) plans to transition approximately **2 million Medi-Cal members with Unsatisfactory Immigration Status (UIS)** from Managed Care Plans to Fee-for-Service system. It's important to note that Medi-Cal FFS system alone is not access. Access requires providers willing to see members, systems that help members navigate care, and accountability for outcomes. Few providers, and even fewer specialists, accept FFS Medi-Cal, creating the risk of coverage without meaningful access, particularly for children, older adults, and people with complex needs.

Table 1 summarizes key differences between the Managed Care and Fee-for-Service models.

Table 1: Key differences between Managed Care and Fee-for-Service Models

Domain	Managed Care	Fee-for-Service
Provider Choice and Access	Members access care through providers in the MCP's network. Members select or are assigned a primary care provider (PCP). MCPs are required to maintain an adequate network to provide medically necessary covered services.	Members may see any provider statewide who is enrolled as a Medi-Cal FFS provider and is accepting Medi-Cal FFS members; no assigned PCP. Access depends on provider participation in FFS, not just coverage.
Care Coordination	PCP is responsible for delivering primary care services, making referrals, leading care management, and coordinating services with a care management team as needed.	Limited centralized care coordination. Some providers may bill for care coordination services. This may affect continuity of care and patient navigation.
Provider Management	MCP manages provider credentialing, authorizations, utilization management, and pays providers.	Providers work directly with DHCS for enrollment, authorizations (when required), and claims payment.
Community Supports & Enhanced Care Management (ECM)	Available through Managed Care.	Not available under FFS. Some members may lose access to services that address social and complex care needs.

While the policy decision has already been made by the state, its operational and strategic implications remain uncertain. The transition may affect healthcare access, provider participation, care coordination, continuity of care, costs, and the long-term financial sustainability of managed

care organizations. For IEHP, understanding these impacts is essential for strategic planning, stakeholder engagement, and future policy discussions.

Problem Statement

What are the tradeoffs and challenges in moving from a Managed Care to a Fee-for-Service model? What are the anticipated impacts of this policy shift, the potential risks, and unintended consequences? What should IEHP prioritize?

- **Impact on IEHP**: What does this shift mean for IEHP **financially** and **operationally** (for example, lost revenue, fixed costs, and its provider network)? What are the potential **risks** and unintended **consequences**?

- **Impact on patients**: Where might patients experience the greatest **loss of access** to care? **Which geographic areas** appear most vulnerable because of limited provider participation?

- **Actions**: What actions should **IEHP prioritize** in the short and long term?

- **Metrics:** What **should IEHP monitor** during and after the transition?

Examples of Background and Data Sources

- IEHP website
  - Fact sheet (October 2026)
  - Governing board reports (including finance committee reports)
- DHCS Policy Guide for UIS

Please refer to the following for additional information and data sources:

1. Video from **Jarrod McNaughton, CEO, IEHP**
2. Slide deck from **Oct 7 Information Session**
3. Slide deck from **Oct 14 Networking Session** with **Jayson Yardley, CEO, Patient Funding Alternatives**
4. Student Guide: **Access to Care Analysis** document
5. Student Guide: **Financial Impacts Modeling** spreadsheet

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The Challenge

Analyze the problem in teams of 3-5 students and prepare a **five-minute video presentation** of your team's insights and recommendations. Focus on executive-level storytelling supported by data, and provide recommendations to the CEO on how IEHP should respond to the policy changes. Due **Monday, November 9, 2026 by 5 pm.**

Finalist teams will be announced on **Friday, November 13, 2026**, and invited to **present on Thursday, November 19, 2026, 1:00 PM – 3:30 PM** (Building 163, Room 1005) at a networking, presentations, and awards event.

Feel free to make reasonable assumptions and create what-if scenarios. Limited use of **Generative AI** is permitted — for example, summarizing articles and policy documents, brainstorming ideas, or getting help with Excel formulas, code, or editing your script. AI-generated numbers you can't trace to a source, and AI voices or avatars standing in for your team's own presentation, are not permitted. Add a final slide listing the AI tool(s) you used and what you used them for. Please remember that you are responsible for the accuracy of your analysis and findings.

Collaborate and enjoy!

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